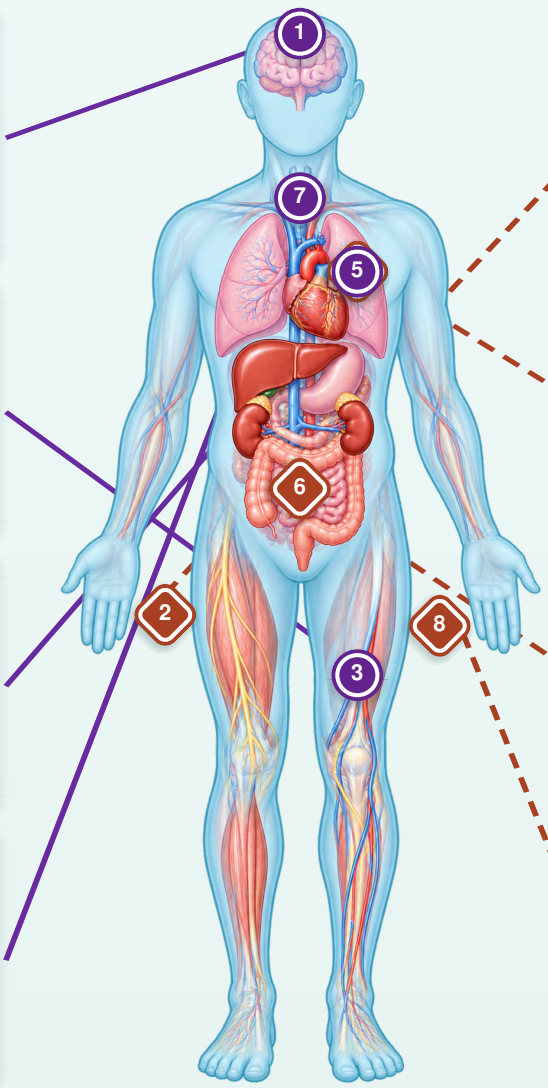
Pembrolizumab 400 mg + weekly carboplatin AUC 1.5 / paclitaxel → dose-dense AC

Phase 1: four 21-day cycles; paclitaxel on Days 1, 8 and 15, carboplatin on all three days. Phase 2: four AC cycles every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 1, 3, 5 and 8; after surgery, every 42 days for five doses. See the whole-treatment map for monitoring and sequencing.

Source reconciled; independent oncology, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Hair / scalp

Paclitaxel

Hair loss is common; ask about support and scalp cooling where suitable.

2 Nerves / nails

Paclitaxel

Report numbness, tingling or painful nails before the next dose.

3 Blood / infection

Carboplatin and paclitaxel

Fever, shivers, unusual bleeding or sudden illness: call immediately.

4 Infusion reaction

Carboplatin or paclitaxel

Tell staff immediately about flushing, chest tightness or breathing difficulty.

5 Breathing

Pembrolizumab

New cough or breathlessness needs prompt assessment.

6 Bowel

Pembrolizumab

Report diarrhoea early, especially with pain, bleeding or poor intake.

7 Hormone glands / energy

Pembrolizumab

Report marked weakness, dizziness, headache or unusual thirst.

8 Skin / liver symptoms

Pembrolizumab

A severe rash, blistering or jaundice needs prompt advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

This printed map shows phase 1. See the next page for all medicines and the online phase-specific maps. Report persistent effects after a phase ends.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Pembrolizumab

- Immune inflammation of bowel, lungs, liver, kidneys or skin
- Hormone-gland problems, sometimes permanent
- Rare serious heart or neurological inflammation
- Infusion reaction
- Effects can first appear after treatment ends

Carboplatin

- Low blood counts, infection and bleeding
- Nausea, vomiting and fatigue
- Allergic reaction, sometimes after previous exposure
- Kidney or electrolyte changes

Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Infusion reaction
- Mouth, bowel, skin and nail effects

Doxorubicin

- Low counts, infection and mouth ulcers
- Nausea, hair loss and fatigue
- Heart damage, sometimes delayed
- Severe tissue injury if the infusion leaks
- Rare later blood cancers

Cyclophosphamide

- Low counts and infection
- Nausea, vomiting and hair loss
- Bladder irritation / bleeding
- Fertility effects and menopause
- Rare later blood cancers

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Separate anti-sickness plans

The platinum/taxane phase and AC need separate prescriptions. Carboplatin/paclitaxel is moderate emetic risk on each weekly treatment day. AC uses its own high-risk plan.

Growth-factor support

granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is prescribed for every dose-dense AC cycle. Ask for the exact injection days and instructions.

Checks before the next dose

full blood count (FBC), renal and liver tests weekly during the carboplatin/paclitaxel phase, then before every AC cycle. Continue the required blood tests before each postoperative pembrolizumab cycle. Keep the thyroid and other immune-monitoring appointments.

After chemotherapy

After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00861/

Sources and review status are available online. Follow your own prescription and hospital instructions.